NUTRITION



Malnutrition is present in approximately 45% of patients at time of admission to hospital. It is often underrecognized and is associated with increased postoperative complications and in-hospital and 30-day mortality. Nutrition risk is also associated with increased length of stay, readmission, and hospital costs. [1-5] Preoperative & early postoperative nutritional intervention are associated with improvements in postoperative complications and mortality. [6]

Screening Tools

The Canadian Nutrition Screening Tool (CNST) is a valid and reliable screening tool to identify those patients at risk of malnutrition in the adult acute care environment. [1]

CANADIAN NUTRITION SCREENING TOOL (CNST)

Name:	Age:	Weight:	Room:

Identify patients who are at risk for malnutrition

	Date:		Date:	
	Admission		Rescreening	
Ask the patient the following questions*	**Yes**	**No**	**Yes**	**No**
Have you lost weight in the past 6 months **WITHOUT TRYING** to lose this weight? If the patient reports a weight loss but gained it back, consider it as NO weight loss.				
Have you been eating less than usual **FOR MORE THAN A WEEK**?				
Two "YES" answers indicate nutrition risk†				

* If the patient is unable to answer the questions, a knowledgeable informant can be used to obtain the information. If the patient is uncertain regarding weight loss, ask if clothing is now fitting more loosely.

Patients at nutrition risk need an assessment to confirm malnutrition

Nutrition screening using a valid tool can generate a significant volume of requests for nutrition evaluation. Subjective Global Assessment (SGA) is a simple and efficient first-line assessment of nutritional status that can be used following a positive screening and to help prioritize cases.

If a patient is malnourished (SGA B or C), an in-depth nutrition assessment, along with treatment, is required by a registered dietitian.

The Canadian Nutrition Screening Tool was rigorously validated and tested for reliability in Canadian hospitals. Non-expert raters completed the tool and it was compared to the SGA conducted by a dietitian or trained nutrition researcher.

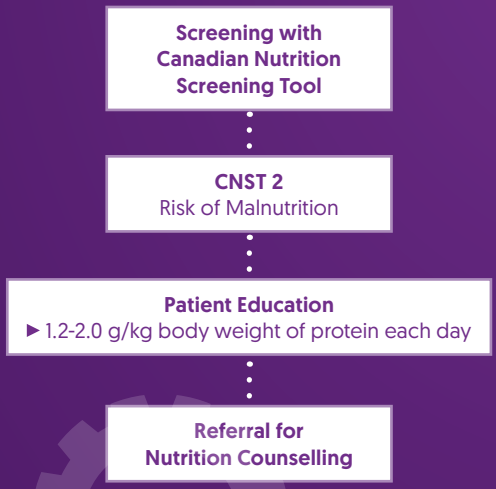
† If a patient is not at risk, rescreen within a week. Only consider weight change in the past week.

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Canadian Nutrition *Society*
Société canadienne *de* nutrition

Canadian Malnutrition Task Force | le Groupe de travail canadien sur la malnutrition

NUTRITION ALGORITHM



Prehabilitation and Optimization Recommendations

Patient Education	⚙ Patients that are at high risk for malnutrition (CNST 2) require dietician assessment. It is essential that the patient is able to meet their energy, carbohydrate, and protein needs prior to surgery. For 2-4 weeks prior to surgery an increase in dietary protein (aiming for 1.2-2 g/kg/day) may be recommended. ⚙ Refer to online patient resources
Referral for Nutrition Counseling	⚙ Dietician to confirm diagnosis of malnutrition and advise dietary plan for optimized nutrition before surgery ⚙ Refer to online provider resources for dietician resources and referral templates